



POLICY

MH Credentialing and Scope of Practice for Neuromodulation Treatments (ECT and rTMS)

Scope (Staff): All NMHS Mental Health (MH) Clinical Staff

Scope (Area): All NMHS MH Clinical Areas

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1. Aim

NMHS, MH services operate in accordance with the outlined scope of practice and credentialing requirements for professionals responsible for the provision of Electroconvulsive Therapy (ECT), Repetitive Transcranial Magnetic Stimulation (rTMS) or other Neuromodulation treatments. The policy outlines the credentialing process for Medical staff as per mandatory policy. The specification of competencies, qualifications and requirements for non-medical staff are outlined and are the responsibility of the operational service.

2. Background

The minimum education and training requirements for medical practitioners participating in ECT, rTMS and/or other Neuromodulation treatments for our service will comply with:

- [Chief Psychiatrist Practice Standards for the Administration of Electroconvulsive Therapy \(2015\).](#)
- [The E.C.T. Guide: The Chief Psychiatrist's Guidelines for the use of Electroconvulsive Therapy in Western Australia \(2006\) The Mental Health Act, 2014](#)

and is informed by:

- The Clinical TMS Society Consensus Recommendations.
- The Small but Critical Professions Project; Neurophysiology Technologists; Office of the Chief Health Professions Officer; Clinical Services and Research Division Western Australian Department of Health (Draft).
- The Royal Australian & New Zealand College of Psychiatrists Position Statement 74 and Professional Practice Guidelines: Electroconvulsive Therapy (ECT).
- The Royal Australian & New Zealand College of Psychiatrists, Position Statement 79 and Professional Practice Guidelines; Repetitive Transcranial Magnetic Stimulation.
- Royal College of Nursing Guidance for ECT, April 2005.
- ECT Accreditation Service (ECTAS) Standards for the administration of ECT: 13th ED, 2016.
- NICE guidance on the use of electroconvulsive therapy (NICE technology appraisal guidance TA59).
- NICE Repetitive transcranial magnetic stimulation for depression; Interventional procedures guidance.

3. Risk

Risk of non-compliance with this policy include:-

- Impact on Patient Quality of Care
- Impact on Patient Safety, rights of dignity. Breach Office of the Chief Psychiatrist Clinical Practice Standards for Care of ECT
- Breach Professional Standards
- Breach of NMHS MH Mission and Values

Culture	☐	Organisational Development	☒
Emergency Management	☐	Policy & Legislative Compliance	☒
Fiscal Responsibility	☐	Quality of Patient Care	☒

Governance & Accountability	☐	Research & Innovation	☐
Information Communications Technology	☐	Staff Safety & Wellness	☐
Facility Management	☐	Workforce Planning	☐
Medical Equipment	☐	Other	☐

4. Definitions

Applicant: A medical practitioner who has submitted an application for credentialing or re-credentialing using [CredWA](#).

Appointing Officer: A senior officer (however titled) of a Health Service Provider who has delegated authority or is otherwise authorised to offer employment or other engagement to a medical practitioner at a Health Care Facility or group of Health Care Facilities. Appointing officers are commonly but not exclusively Principal Administrators.

Credentialing: The formal process used to verify the qualifications, experience, professional standing and other relevant professional attributes for the purpose of forming a view about their competence, performance and professional suitability to provide a safe, high quality healthcare service within specific environments.

ECT Interventionist: A Consultant Psychiatrist who administers ECT to a patient.

Scope of clinical practice: The extent of an individual practitioner's approved clinical practice within a particular organisation (NMHS) based on an individual's credentials, competence, performance and professional suitability and the needs and capability of the organisation to support the practitioners scope of clinical practice.

5. Principles

The key principles which underpin credentialing and defining scope of clinical practice processes include ([MP 0084/18](#)):

- *Patient safety* – by ensuring medical practitioners practice within their capability of education and training and within the capacity of the Health Care Facility in which they are working;
- *Consistency* – by ensuring alignment with recognised National Safety and Quality Standards; and
- *Natural justice and procedural fairness* – by ensuring credentialing and scope of clinical practice processes are underpinned by natural justice and procedural fairness.

6. Process

- NMHS MH managers are responsible for ensuring medical practitioners are registered and credentialed to perform their duties. Managers will obtain evidence as per requirements listed below to suffice application to the NMHS Credentialing Committee. The Director of

Clinical Services, NMHS MH is a member of the NMHS Credentialing Committee and can provide provisional credentialing awaiting review by the committee.

- A scope of clinical practice is part of the credentialing process and defines the clinical practice that a health practitioner is permitted to conduct at a particular health care facility. The scope of clinical practice is based on an individual's qualifications, competence, performance and suitability as well as a health service's capability to support the scope of practice and service role required.
- An applicant (Consultant Psychiatrist) supplies the required evidence to their line manager who will liaise with NMHS Human Resources Consultant. The applicant should state the scope of clinical practice as being either for Adult, Older Adult or Youth – 1, 2 or 3 can be selected.
- In addition to the above, ECT must be selected for a Consultant Psychiatrist to be able to practice ECT as an ECT Interventionist in addition to the provision of evidence listed below.
- For ECT Credentialing evidence of an applicant meeting the criteria e.g. Qualification attainment, must be uploaded into CredWA. The process is for the applicant to provide this evidence to the Head of Clinical Service and Operational Manager by request, who will then check and provide to Human Resources for approval. Once approved it will be uploaded to [CredWA](#) for consideration by the Director of Clinical Services and submission to the Credentialing Committee. Human Resources will advise the Head of Clinical Service/Operational Manager when re-credentialing is required.
- Scope of Practice and Credentialing applies to Medical staff for a period of 5 years before re-credentialing is required.

7. Medical Staff Credentialing

Initial Credentialing for ECT Interventionist Practice for a period of 5 years

It is recommended that clinical privileges for the practice of ECT should be restricted to RANZCP certified psychiatrists or equivalent trained in modern ECT practice consistent with the, [Chief Psychiatrist's Practice Standards for the Administration of Electroconvulsive Therapy](#).

It is recommended that clinical privileges for the authorisation for administration of rTMS should be restricted to RANZCP certified psychiatrists or equivalent trained in modern rTMS practice.

A Consultant Psychiatrist or Psychiatrist Registrar who wishes to practice unsupervised ECT should first be able to demonstrate:

- Fulfilment of all of the requirements for the conference of fellowship of the College qualification as stipulated by RANZCP or equivalent.
- Proficient administration of a minimum 10 ECT treatments under supervision of a RANZCP or equivalent approved fellow over the last 12 months.
- Attendance at a recognised ECT course.
- Attendance at a local site orientation as provided by the ECT coordinator

A rTMS Consultant Psychiatrist who wishes to practice rTMS should first be able to demonstrate:

- Fulfilment of all of the requirements for the conference of fellowship of the

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- College qualification as stipulated by RANZCP or equivalent body.
 - Fulfilment of all of the requirements for Full (International Full) membership of the Clinical TMS Society (Trade Association, USA), or the Australian national successor body as developed.
 - Management of a minimum 10 rTMS treatments under supervision of a RANZCP or equivalent approved fellow who is proficient rTMS practitioner over the last 12 months.
 - Attendance at a recognised rTMS course or training program.
 - Attendance at a local site orientation as provided by the rTMS director/coordinator.

8. Renewal of Re-Credentialing

The requirements for ongoing approval to practice as an ECT Interventionist:

- Continued fulfilment of all of the requirements for the conference of fellowship of the College qualification as stipulated by RANZCP or equivalent body.
- Performing or personally supervising a minimum of 10 ECT treatments per annum).
- The ability to demonstrate annual activity attributable to ongoing professional development in the area of ECT. This can include:
 - conference attendance;
 - authoring or preparation of peer-reviewed publications;
 - teaching or demonstration;
 - visiting and or participation in ECT administration at other sites.
- Continued fulfilment of all of the requirements for the conference of fellowship of the College qualification as stipulated by RANZCP or equivalent body.
- Proficient management of a minimum of 10 rTMS treatment courses per annum.
- The ability to demonstrate annual activity attributable to ongoing professional development in the area of rTMS. This can include:
 - conference attendance;
 - authoring or preparation of peer-reviewed publications;
 - teaching or demonstration;
 - visiting and or participation in rTMS administration at other sites.
- Attendance at a recognised at a CTMS affiliated or equivalent site training program.
- Annual attendance at a local site orientation as provided by the ECT coordinator.

9. Operational Staff

The section below outlines the core competencies, training, skill and knowledge standard for practitioners in the field of ECT and rTMS. There is no formalised credentialing process undertaken, instead the Sir Charles Gairdner Hospital Mental Health Service maintains responsibility for ensuring all practitioners meet the requirements outlined before undertaking provision of clinical treatment.



9.1 ECT Coordinator

Individuals wishing to fulfil the role of ECT Coordinator must show the following:

- Registration with the Australian Health Practitioner Regulation Agency (AHPRA), as an Enrolled or Registered Nurse.
- Attendance at a recognised ECT course every five years from the period of initial attendance.
- At least one year of experience in assisting in the process of ECT administration (See 'Nurse involved in administration section' for more information).
- Observation and assessment of three full rounds of treatment of ECT by an existing ECT coordinator and/or ECT Director.
- Participation in at least 40 ECT procedures in the previous 12 months.
- Exhibits knowledge of current national (Royal Australian New Zealand College of Psychiatrists) and state operational practice guidelines (The Chief Psychiatrists Guidelines for the use of Electroconvulsive therapy in Western Australia 2006, Chief Psychiatrist Practice Standards for the Administration of Electroconvulsive Therapy).
- Annual attendance at a local site orientation as provided by the current (or previous) ECT coordinator.

The minimum requirements for ongoing approval to practice are:

- Attendance at a recognised ECT training program every five years from the period of initial attendance.
- Personally supervising a minimum of 40 ECT treatments per annum.
- The ability to demonstrate ongoing professional development in the area of ECT (this can include conference attendance, the authoring or peer-reviewed publications, visiting and or participation in ECT administration at other sites and facilities with the intent on augmenting skills).

9.2 rTMS Coordinator

rTMS Coordinator: Credentialed to conduct authorised administration of rTMS treatment course:

- Fulfilment of all of the requirements for Full (International Full) membership of the Clinical TMS Society (Trade Association, USA), or the Australian national successor body as developed.
- Registration with the Association of Neurophysiological Technologists of Australia Inc and associated scope of practice.
- Attendance at a recognised rTMS course at a CTMS affiliated or equivalent site training program.
- At least one year of experience in assisting in the process of rTMS administration.
- Participation in at least 10 rTMS treatment courses in the previous 12 months.
- Exhibits knowledge of current national (Royal Australian New Zealand College of Psychiatrists) and CTMS operational practice guidelines.



The minimum requirements for ongoing approval to practice are:

- Attendance at a recognised TMS training program every five years from the period of initial attendance.
- Personally supervising a minimum of 10 rTMS treatment courses per annum.
- The ability to demonstrate ongoing professional development in the area of TMS (this can include conference attendance, the authoring or peer-reviewed publications, visiting and or participation in rTMS administration at other sites and facilities with the intent on augmenting skills).

9.3 Nurse involved in Administration

In general, nursing staff can work with rTMS, if they are accepted in the ANTA association under a Neurophysiology Technologists scope of practice (see below).

9.4 ECT nurse

Individuals wishing to fulfil the role of ECT nurse must show the following:

- Registration with the Australian Health Practitioner Regulation Agency (AHPRA), as an Enrolled or Registered Nurse.
- Exhibits knowledge of current national (Royal Australian New Zealand College of Psychiatrists) and state operational practice guidelines (The Chief Psychiatrists Guidelines for the use of Electroconvulsive therapy in Western Australia 2006, Chief Psychiatrist Practice Standards for the Administration of Electroconvulsive Therapy).
- Annual attendance at a local site orientation as provided by the (current or previous) ECT coordinator.
- Attendance at a recognised ECT course.
- Review and Observation of satisfactory practice by the ECT coordinator for the first three months.

9.5 rTMS Technologist

Technologist involved administering authorised rTMS scope of practice and credentialing requirements are as follows:

- Registration with the Association of Neurophysiological Technologists of Australia Inc (ANTA).
- Attendance at a recognised rTMS course at a CTMS affiliated or equivalent sitetraining program.
- At least one year of experience in assisting in the process of rTMS administration.
- Participation in at least 10 rTMS treatment courses in the previous 12 months.
- Exhibits knowledge of current national (Royal Australian New Zealand College of Psychiatrists) and CTMS operational practice guidelines.

Note: Enrolled or Registered Nursing staff conducting rTMS should also be registered (Associated Member) with the ANTA.



The minimum requirements for ongoing approval to practice are:

- Registration with the Australian Health Practitioner Regulation Agency (AHPRA), as an Enrolled or Registered Nurse.
- Exhibits knowledge of current national (Royal Australian New Zealand College of Psychiatrists) and state operational practice guidelines (The Chief Psychiatrists Guidelines for the use of Electroconvulsive therapy in Western Australia 2006, Chief Psychiatrist Practice Standards for the Administration of Electroconvulsive Therapy).
- Annual attendance at a local site orientation as provided by the (current or previous) ECT coordinator.
- Registration with the Association of Neurophysiological Technologists of Australia (ANTA) Inc.
- Attendance at a recognised TMS training program every five years from the period of initial attendance.
- Annual attendance at a local site orientation as provided by the (current or previous) rTMS coordinator.

9.6 Stage 1 Recovery Nurse – ECT only

Individuals wishing to fulfill the role of recover nurse must show the following:

- Registration with the Australian Health Practitioner Regulation Agency (AHPRA), as a Registered Nurse.
- Attain post-graduate specialist training to meet the standards of the Australian College of Recovery Nursing (ACORN) or equivalent - experience as a Post Anaesthetic Care Unit Nurse or completion of an airway management course or higher at a minimum.

9.7 Stage 2 Recovery Nurse – ECT only

Individuals wishing to fulfill the role of recovery nurse must show the following:

- Registration with the Australian Health Practitioner Regulation Agency (AHPRA), as an Enrolled or Registered Nurse.
- Completion of an airway management course or higher at a minimum ECT Education Session covering the role and responsibilities of the nurse caring for the ECT patient.
- Annual attendance at a local site orientation as provided by the (current or previous) ECT coordinator.
- Review and observation of satisfactory practice by the ECT coordinator for the first three months.

9.8 Ward Nurses

The minimum requirements for ongoing approval to practice are:

- Registration with the Australian Health Practitioner Regulation Agency (AHPRA), as an Enrolled or Registered Nurse.
- ECT Education Session covering the role and responsibilities of the nurse caring for the ECT patient.



- Annual attendance at a local site orientation as provided by the (current or previous) ECT coordinator.
- Review and Observation of satisfactory practice by CNS/SDN

9.9 Transport and Escorted Nurse

Individuals wishing to fulfil the role of recovery nurse much show the following:

- Registration with the Australian Health Practitioner Regulation Agency (AHPRA), as an Enrolled or Registered Nurse.
- The nurse escorting the patient must have the expertise appropriate to patient acuity/specific condition to provide appropriate level of care
- Review and Observation of satisfactory practice by a CNS/SDN.

The minimum requirements for ongoing approval to practice are:

- Registration with the Australian Health Practitioner Regulation Agency (AHPRA), as an Enrolled or Registered Nurse.
- The nurse escorting the patient must have the expertise appropriate to patient acuity/specific condition to provide appropriate level of care.
- Review and Observation of satisfactory practice by a CNS/SDN.

10. Compliance and Evaluation

The SCGH Mental Health Service are responsible for keeping a log of credentialed staff and liaison with NMHS, MH Human Resources to ensure that all Consultant Psychiatrist credentialing and scope of practice documents are loaded onto [CredWA](#).

SCGH Mental Health Service will maintain a log of operational staff defined as compliant with the standards outlined for the provision of ECT and rTMS.

Related internal policies, procedures and guidelines

[MP0084/18 Credentialing and Defining Scope of Clinical Practice Policy](#)
[Chief Psychiatrist's Practice Standards for Administration of Electroconvulsive Therapy](#)
[CredWA](#)



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NSQHS Standards Applicable:	☒  Std 1: Clinical Governance ☒  Std 2: Partnering with Consumers ☒  Std 3: Preventing and Controlling Healthcare Associated Infection ☒  Std 4: Medication Safety		☒  Std 5: Comprehensive Care ☒  Std 6: Communicating for Safety ☒  Std 7: Blood Management ☒  Std 8: Recognising and Responding to Acute Deterioration		
National Standards for Mental Health Services	☒ Std 1: Rights and Responsibilities ☐ Std 2: Safety ☐ Std 3: Consumer and Carer Participation ☐ Std 4: Diversity Responsibility ☐ Std 5: Promotion and Prevention ☐ Std 6: Consumers ☐ Std 7: Carers ☐ Std 8: Governance, leadership and management		☐ Std 9: Integration ☐ Std 10: Delivery of Care ☐ 10.1 Supporting Recovery ☐ 10.2 Access ☐ 10.3 Entry ☐ 10.4 Assessment and Review ☐ 10.5 Treatment and Support ☐ 10.6 Exit and Re-entry		
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